

Correction

Correction to: 2017 ACC/AHA/AAPA/ABC/ACPM/AGS/APhA/ASH/ASPC/NMA/PCNA Guideline for the Prevention, Detection, Evaluation, and Management of High Blood Pressure in Adults: Executive Summary: A Report of the American College of Cardiology/American Heart Association Task Force on Clinical Practice Guidelines

In the article by Whelton et al, “2017 ACC/AHA/AAPA/ABC/ACPM/AGS/APhA/ASH/ASPC/NMA/PCNA Guideline for the Prevention, Detection, Evaluation, and Management of High Blood Pressure in Adults: Executive Summary: A Report of the American College of Cardiology/American Heart Association Task Force on Clinical Practice Guidelines,” which published ahead of print on November 13, 2017, and appeared in the June 2018 issue of the journal (*Hypertension*. 2018;71:1269–1324. DOI: 10.1161/HYP.0000000000000066), several corrections were needed.

1. On page 1269, in the author list, “FACC” has been removed after Dr. Giddings’ name.
2. On page 1270, in the Table of Contents and on page 1295, the 9.5. heading read, “9.5. Peripheral Arterial Disease.” It has been updated to read, “9.5. Peripheral Artery Disease.” It has also been updated on page 1303 in Table 22 and on page 1304 in Table 23.
3. On page 1276, in the footnote for Table 5, second paragraph, last sentence, the reference 3 citation has been removed from the end of the sentence.
4. On page 1278, the heading “4.3. Ambulatory BP Monitoring” has been deleted. The heading “4.4. Masked and White Coat Hypertension” now reads, “4.3. Masked and White Coat Hypertension.”
5. On page 1279, Figure 1, bottom row, needed the following changes:
 - The third box from the left (“**Masked Hypertension**”) was labeled as Class IIb. It has been updated to Class IIa. The shading of the box has been updated also.
 - The fourth box from the left (“**Elevated BP**”) was labeled as Class IIb. It has been updated to Class IIa. The shading of the box has been updated also.
6. On page 1280, Figure 2, bottom row, the second box from the left read, “Continue current therapy (Class IIa).” It has been updated to read, “Continue current therapy”; the Class of Recommendation color has been removed.
7. On page 1284, in Table 14, the text in the first column, last row read, “Angiogenesis inhibitor (e.g., bevacizumab) and tyrosine kinase inhibitors (eg, sunitinib, sorafenib).” It has been updated to read, “Angiogenesis inhibitor (eg, bevacizumab) and tyrosine kinase inhibitors (eg, sunitinib, sorafenib).”
8. On page 1287, legend to Figure 4, the third sentence, read, “*Using the ACC/AHA Pooled Cohort Equations (57).” It has been updated to read, “*Using the ACC/AHA Pooled Cohort Equations.^{S8.1.2-14,S8.1.2-15}”
9. On page 1287, the section “8.1.3. Follow-Up After Initial BP Evaluation” recommendation table title read, “Recommendations for Follow-Up After Initial BP Elevation.” It has been updated to read, “Recommendations for Follow-Up After Initial BP Evaluation.”
10. On pages 1288–1290, in “Table 18. Oral Antihypertensive Drugs,” the following updates have been made:
 - In the “Primary agents” section, the first row “Thiazide or thiazide-type diuretics,” the “Metolazone” section, the “Usual Dose, Range (mg/d)” column read, “2.5–10.” It has been updated to read, “2.5–5.”

- In the “Primary agents” section, the second row “ACE inhibitors,” the “Ramipril” section, the “Usual Dose, Range (mg/d)*” column read, “2.5–10.” It has been updated to read, “2.5–20.”
- In the “Primary agents” section, the fourth row “CCB—dihydropyridines,” the “Felodipine” section, the “Usual Dose, Range (mg/d)*” column read, “5–10.” It has been updated to read, “2.5–10.”
- In the “Primary agents” section, the fourth row “CCB—dihydropyridines,” the “Nicardipine SR” section, the “Usual Dose, Range (mg/d)*” column read, “5–20.” It has been updated to read, “60–120.” The “Daily Frequency” column read, “1.” It has been updated to read, “2.”
- In the “Primary agents” section, the fourth row “CCB—dihydropyridines,” the “Nifedipine LA” section, the “Usual Dose, Range (mg/d)*” column read, “60–120.” It has been updated to read, “30–90.”
- In the “Primary agents” section, the fourth row “CCB—dihydropyridines,” the “Nisoldipine” section, the “Usual Dose, Range (mg/d)*” column read, “30–90.” It has been updated to read, “17–34.”
- In the “Primary agents” section, the fifth row “CCB—nondihydropyridines,” the “Diltiazem SR” section has been removed.
- In the “Primary agents” section, the fifth row “CCB—nondihydropyridines,” the “Diltiazem ER” section, the “Usual Dose, Range (mg/d)*” column read, “120–480.” It has been updated to read, “120–360.”
- In the “Primary agents” section, the fifth row “CCB—nondihydropyridines,” the “Verapamil IR” section, the “Usual Dose, Range (mg/d)*” column read, “40–80.” It has been updated to read, “120–360.”
- In the “Primary agents” section, the fifth row “CCB—nondihydropyridines,” the “Verapamil SR” section, the “Usual Dose, Range (mg/d)*” column read, “120–480.” It has been updated to read, “120–360.”
- In the “Primary agents” section, the fifth row “CCB—nondihydropyridines,” the “Verapamil-delayed onset ER” section, the “Drug” column read, “Verapamil-delayed onset ER (various forms).” It has been updated to read, “Verapamil-delayed onset ER.”
- In the “Primary agents” section, the fifth row “CCB—nondihydropyridines,” the “Verapamil-delayed onset ER” section, the “Usual Dose, Range (mg/d)*” column read, “100–480.” It has been updated to read, “100–300.”
- In the “Secondary agents” section, the first row “Diuretics—loop,” the “Bumetanide” section, the “Usual Dose, Range (mg/d)*” column read, “0.5–4.” It has been updated to read, “0.5–2.”
- In the “Secondary agents” section, the third row “Diuretics—aldosterone antagonists,” the “Eplerenone” section, the “Daily Frequency” column read, “12.” It has been updated to read, “1 or 2.”
- In the “Secondary agents” section, the fourth row “Beta blockers—cardioselective,” the “Atenolol” section, the “Daily Frequency” column read, “12.” It has been updated to read, “2.”
- In the “Secondary agents” section, the fourth row “Beta blockers—cardioselective,” the “Metoprolol tartrate” section, the “Usual Dose, Range (mg/d)*” column read, “100–400.” It has been updated to read, “100–200.”
- In the “Secondary agents” section, the sixth row “Beta blockers—noncardioselective,” the “Propranolol IR” section, the “Usual Dose, Range (mg/d)*” column read, “160–480.” It has been updated to read, “80–160.”
- In the “Secondary agents” section, the sixth row “Beta blockers—noncardioselective,” the “Propranolol LA” section, the “Usual Dose, Range (mg/d)*” column read, “80–320.” It has been updated to read, “80–160.”
- In the “Secondary agents” section, the seventh row “Beta blockers—intrinsic sympathomimetic activity,” the “Carteolol” section has been deleted.
- In the “Secondary agents” section, the tenth row “Alpha-1 blockers,” the “Doxazosin” section, the “Usual Dose, Range (mg/d)*” column read, “1–8.” It has been updated to read, “1–16.”
- In the “Secondary agents” section, the penultimate row, the “Class” column read, “Central alpha₁-agonist and other centrally acting drugs.” It has been updated to read, “Central alpha₂-agonist and other centrally acting drugs.”
- In the “Secondary agents” section, the last row “Direct vasodilators,” the “Hydralazine” section, the “Usual Dose, Range (mg/d)*” column read, “250–200.” It has been updated to read, “100–200.”

11. On page 1291, the heading “8.3.2. Monitoring Strategies to Improve Control of BP in Patients on Drug Therapy for High BP” has been renumbered as “8.2.2. Monitoring Strategies to Improve Control of BP in Patients on Drug Therapy for High BP.”
12. On page 1295, Figure 9, fourth row, the first box on the left read, “Aim for BP <140/90 mm Hg (Class IIb).” It has been updated to read, “Aim for BP <130/80 mm Hg (Class IIb).”
13. On page 1304, Table 23, in the “Specific comorbidities” section, the penultimate row “Secondary stroke prevention (lacunar)” has been deleted.
14. On page 1304, in the “Presidents and Staff” section for the American Heart Association, Ms. Hundley’s title is updated to read, “Manager, Production and Operations.”
15. On page 1310, in the References for section “8.1.2. BP Treatment Threshold and the Use of CVD Risk Estimation to Guide Drug Treatment of Hypertension,” the following changes were made:
 - A reference was added: “S8.1.2-14. Goff DC Jr, Lloyd-Jones DM, Bennett G, et al. 2013 ACC/AHA guideline on the assessment of cardiovascular risk: a report of the American College of Cardiology/American Heart Association Task Force on Practice Guidelines. *Circulation*. 2014;129(suppl 2):S49–73.”
 - Reference “14. Stone NJ, Robinson JG, Lichtenstein AH, et al. 2013 ACC/AHA guideline on the treatment of blood cholesterol to reduce atherosclerotic cardiovascular risk in adults: a report of the American College of Cardiology/American Heart Association Task Force on Practice Guidelines. *Circulation*. 2014;129(suppl 2):S1-45” has been renumbered as “S8.1.2-15.”
16. On page 1317, in Appendix 1, for Dr. Carey, column 2 “Employment” read, “University of Virginia—Dean Emeritus and University Professor, Department of Medicine.” It has been updated to read, “University of Virginia School of Medicine—Dean, Emeritus, and Professor of Medicine.” Column 6 “Personal Research” read, “None.” It has been updated to read, “Daiichi Sankyo Inc†”
17. On page 1318, in Appendix 1, for Dr. Ovbiagele, column 4, “Speakers Bureau,” read, “None.” It has been updated to read, “Boehringer Ingelheim Korea Ltd.”
18. On page 1318, in Appendix 1, for Dr. Wright, column 4, “Speakers Bureau,” read, “None.” It has been updated to read, “Amgen†.”
19. On page 1318, in the Appendix 1 footnotes, the first paragraph has been deleted. It read:

This table represents the relationships of committee members with industry and other entities (RWI) that are considered relevant to this document. Although most ACC/AHA guideline writing committees are constituted such that no more than half the members may have relevant RWI for 1 year before and during development of the guideline, rules for the prevention guidelines require that no members have relevant RWI from 1 year before appointment until 1 year after publication of the guideline. Members’ RWI were reviewed and updated at all meetings and conference calls of the writing committee during the document development period. The complete ACC/AHA policy on RWI is available at <http://www.acc.org/guidelines/about-guidelines-and-clinical-documents/relationships-with-industry-policy>.

It has been updated with 2 new paragraphs to read:

This table represents the relationships of committee members with industry and other entities that were determined to be relevant to this document. These relationships were reviewed and updated in conjunction with all meetings and/or conference calls of the writing committee during the document development process. The table does not necessarily reflect relationships with industry at the time of publication. A person is deemed to have a significant interest in a business if the interest represents ownership of $\geq 5\%$ of the voting stock or share of the business entity, or ownership of $\geq \$5000$ of the fair market value of the business entity; or if funds received by the person from the business entity exceed 5% of the person’s gross income for the previous year. Relationships that exist with no financial benefit are also included for the purpose of transparency. Relationships in this table are modest unless otherwise noted.

According to the ACC/AHA, a person has a *relevant* relationship IF: a) the *relationship or interest* relates to the same or similar subject matter, intellectual property or asset, topic, or issue addressed in the *document*; or b) the *company/entity* (with whom the relationship exists) makes a drug, drug class, or device addressed in the *document*, or makes a competing drug or device addressed in the *document*; or c) the *person or a member of the person’s*

household, has a reasonable potential for financial, professional or other personal gain or loss as a result of the issues/content addressed in the *document*.

20. On page 1318, in the Appendix 1 footnotes, “†Significant relationship.” has been added.
21. In the Comprehensive RWI Data Supplement for the writing committee, the following updates have been made:
 - For Dr. Carey, column 2 “Employment” read, “University of Virginia—Dean Emeritus and University Professor, Department of Medicine.” It has been updated to read, “University of Virginia School of Medicine—Dean, Emeritus, and Professor of Medicine.” Column 6, “Personal Research” read, “None.” It has been updated to read, “Daiichi Sankyo Inc*.”
 - For Dr. Ovbiagele, column 4, “Speakers Bureau” read, “None.” It has been updated to read, “Boehringer Ingelheim Korea Ltd.”
 - For Dr. Wright, column 4, “Speakers Bureau” read, “None.” It has been updated to read, “Amgen*.”

These corrections have been made to the print version and the current online version of the article, which is available at <http://hyper.ahajournals.org/lookup/doi/10.1161/HYP.000000000000066>.